

and edema. The iris was discolored because of layering of a hyphema, and the right pupil showed an afferent defect. Intraocular pressure was normal. The fundus of the right eye showed the retinal vessels perfusing the retina, and there was intraretinal edema that involved the posterior pole, excluding the macula. The left eye was normal. No evidence of emboli, hemorrhage, or optic disk drusen was found. Because the retina was being perfused, it was elected not to institute treatment.

Results of cardiac evaluation with echocardiogram, hemoglobin electrophoresis, and coagulation studies were unremarkable. The patient did not have migraine, and she did not use oral contraceptives.

Twenty hours later the patient had visual acuity of R.E.: no light perception and L.E.: 20/20. The afferent pupillary defect and retinal edema persisted. Three months later, visual acuity remained no light perception in the right eye. The retinal edema had resolved, and there was atrophy of the optic nerve (Fig. 2).

Reports of blindness after general anesthesia is uncommon,² especially in children. Hollenhorst, Svein, and Benoit³ described eight patients, ranging in age from 29 to 50 years, who lost vision after having undergone a neurosurgical procedure requiring a posterior approach. As in our case, the patients' faces were cushioned in well-padded headrests. Two of the patients had hypotensive episodes. At the conclusion of the procedure, visual acuity of hand motions in the affected eye and retinal edema with afferent pupillary defects were consistent findings in these two patients. Final visual acuity ranged from 20/15 to no light percep-

tion, with visual acuity of hand motions or worse in five patients. In an effort to elucidate the cause of the artery occlusion, Hollenhorst, Svein, and Benoit³ performed studies on primates. They concluded that the common denominator when vision was lost was application of pressure to the eyeball. They were unable to conclude that shock was a prerequisite for this condition.

Because of the frequency of spinal fusion in children and the need for early recognition and treatment to prevent loss of vision, we encourage careful monitoring of the face, protection of the eye and orbits during these operations, and prompt consultation if injury is suspected.

References

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Management of Exudative Retinal Detachment After Plaque Therapy for Intraocular Melanoma

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Progressive massive exudative retinal detachment that occurs as a complication of ocular episcleral plaque radiotherapy for choroidal or ciliary body melanoma^{1,2} commonly leads to total loss of vision in the affected eye and occasionally prompts the surgeon to recommend enucleation.³ Based on previous experi-

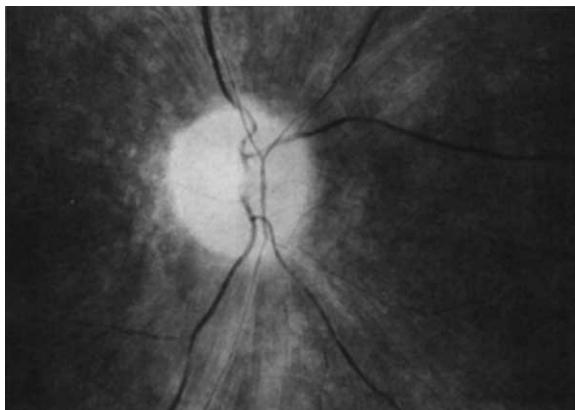


Fig. 2 (Locastro, Novak, and Biglan). The optic nerve of the right eye shows early signs of atrophy three weeks after the operation.